

Title

Patient and provider experiences with community health worker-led remote hypertension management in rural sub-Saharan Africa

Abstract

Background: Task-shifting hypertension care to community health workers (CHWs) using remote clinical oversight may improve rural access. We explored stakeholder perceptions of CHW-facilitated telehealth versus clinic-based care. **Methods:** A qualitative substudy nested within the SEARCH Sapphire randomized trial enrolled 40 hypertensive patients and 15 healthcare providers in rural Kenya and Uganda. Audio-recorded semi-structured interviews explored experiences with community-based hypertension management, care preferences, and integration of remote consultation into clinical and community workflows. Data were analyzed thematically. **Results:** Overall, patients in the CHW-remote arm reported satisfaction with community-based care delivery, citing reduced transportation expenses and improved accessibility for those with mobility limitations or distant clinic residence. Patients perceived CHWs as appropriate for routine management and valued clinician involvement in decision-making. Providers noted telehealth consultation quality comparable to in-person care for stable hypertension but acknowledged need for clinic-based evaluation for complex cases. Successful models incorporated clear communication protocols among patients, CHWs, and clinicians. **Conclusions:** Both patients and providers expressed acceptability of CHW-coordinated remote clinical care as a viable approach to extend hypertension management in resource-limited rural settings, emphasizing the importance of transparent communication and integrated clinical oversight.